



15th April 2025

Mr Oliver Yerbury-Hodgson
Sent Via Email

Ref no: PR7157976

Dear Mr Oliver Yerbury-Hodgson

Recommendation to Orthopaedics

Following your Physiotherapy assessment/treatment, we feel the best course of action is for you to see a specialist, who can decide whether they feel further investigations are needed and discuss any alternative options with you.

You will need to discuss this with your insurance provider to ensure you have the appropriate cover for this service. If you are covered, they will likely provide you an authorisation code and a list of consultants that you can choose from.

Therapist Name: Umm-e-Aiman Khan

HCPC Number: PH138171

Summary:

29 years-old Male presents with a long-standing history of right ankle pain, initially triggered during a hiking trip approximately two years ago. Since the onset, symptoms have been intermittent, primarily aggravated by walking. Recently, the patient has noted a recurrence of symptoms, with increased discomfort over the past month, particularly following awkward positioning and limited running activity. The patient is unsure if the recent discomfort is directly related to their recent running. Pt reports mild morning stiffness in the affected ankle, which resolves with walking. The pain intensity is reported to be a maximum of 3-4/10 on the Numeric Pain Rating Scale (NPRS) during activity and 0/10 at rest. The patient denies any other systemic symptoms such as night pain, weight loss, or neurological deficits, and there is no history of any trauma or red flags. The patient demonstrates normal performance in Single Leg Stand and Squat, completing both movements without difficulty. Heel Raise is performed without issues; however, the patient reports a need to engage the right side more actively. Range of Motion (ROM) is full in all directions with no limitations. On palpation, there is slight tenderness over the medial malleolus, lateral malleolus, and midfoot area. Mild tenderness is noted on palpation around the Achilles tendon, with swelling at lateral boarder of foot, nil bruising, or neurological involvement. Lower extremity strength is intact, and there is no indication of inflammation or significant dysfunction. Despite treatment over period of 23 weeks pain is still present and its plateaued now. Treatment included Dstm, mobilization and combination of strength and stretching exercises.



Despite ongoing treatment, the patient has experienced limited improvement over the past 6 months. Pain levels remain consistent, with no significant reduction in symptoms. Given the lack of progress and plateau in the treatment response, referral to podiatrist is recommended to further assess and manage the condition, as conservative measures have proven insufficient.

Yours sincerely,

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